

Background

- Mavacamten is a cardiac myosin inhibitor approved for the treatment of symptomatic New York Heart Association class II-III obstructive hypertrophic cardiomyopathy.
- In clinical trials up to 6% of patients receiving mavacamten experienced left ventricular systolic dysfunction.
- To mitigate the risk of clinical heart failure and drug-drug interactions mavacamten was approved with a Risk Evaluation and Mitigation Strategy (REMS) program.
- The REMS program requires clinician education, patient education and close patient monitoring.
- In November 2023, the integrated health system specialty pharmacy (HSSP) began dispensing mavacamten.
- Both clinical and dispensing workflows were optimized with a goal of 100% compliance with all REMS requirements.

Objective

- To evaluate mavacamten clinical outcomes and REMS compliance at the HSSP one year post implementation of an optimized clinical and dispensing workflow.

Methods

This is a retrospective chart review of patients who received mavacamten from November 1, 2023, to September 30, 2024.

Patients receiving mavacamten with a diagnosis of obstructive hypertrophic cardiomyopathy were enrolled in the Cardiomyopathy disease program.

The primary outcome included evaluating adherence (MPR and PDC) of mavacamten for patients filling at the HSSP and the yearly audit compliance with the REMS program.

A secondary outcome evaluated was patient reported quality of life.

Results

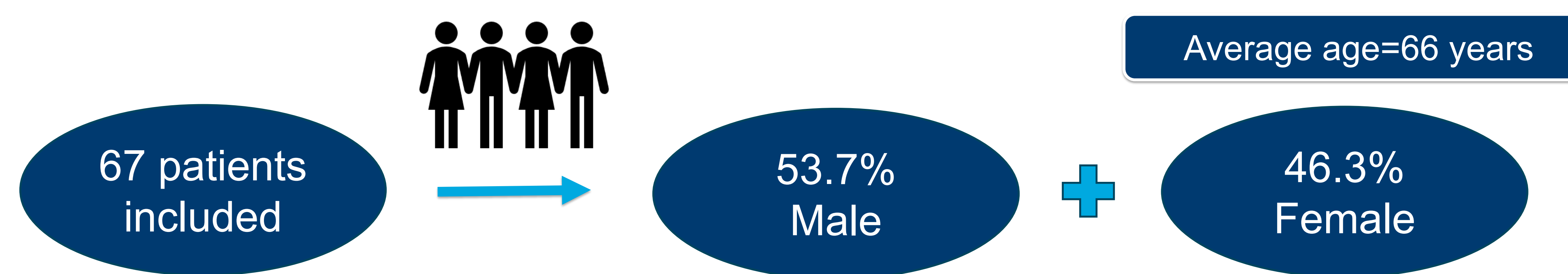


Figure 1: Medication Possession Ratio

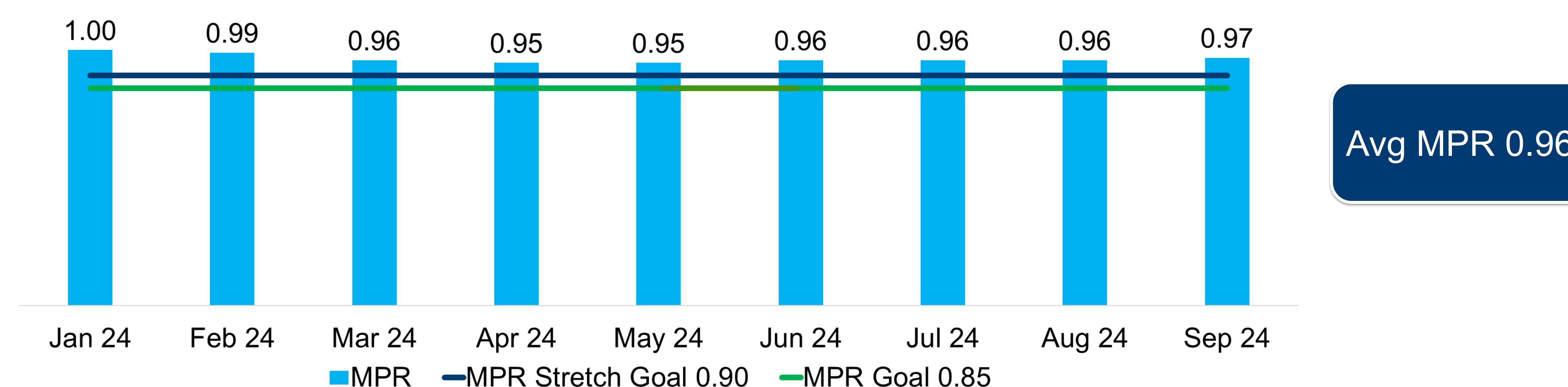


Figure 2: Proportion of Days covered

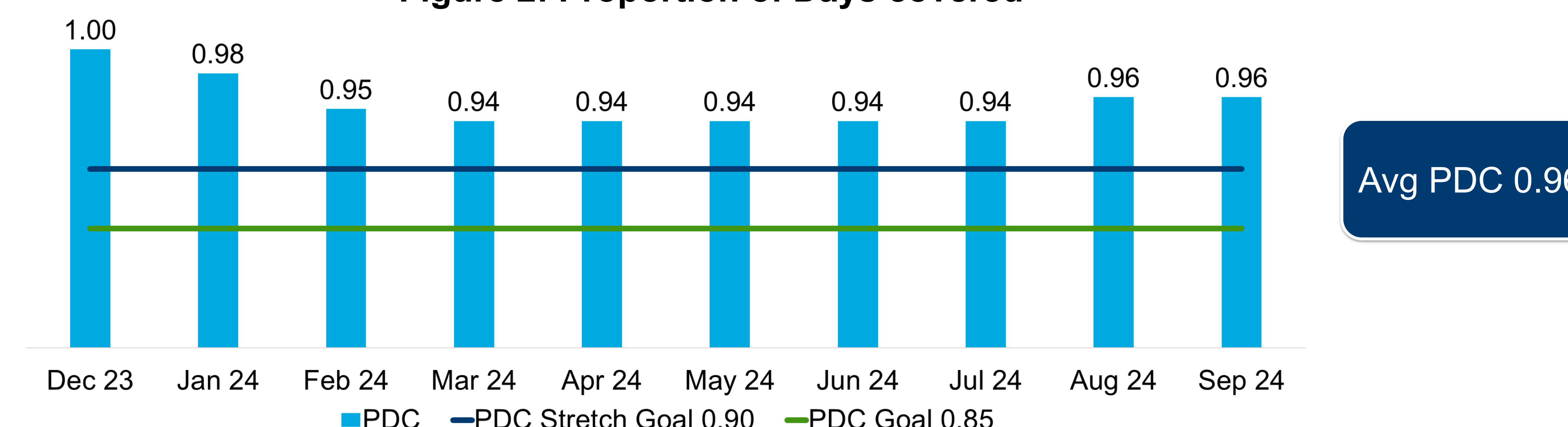


Figure 3: Manufacturer REMS Audit Compliance

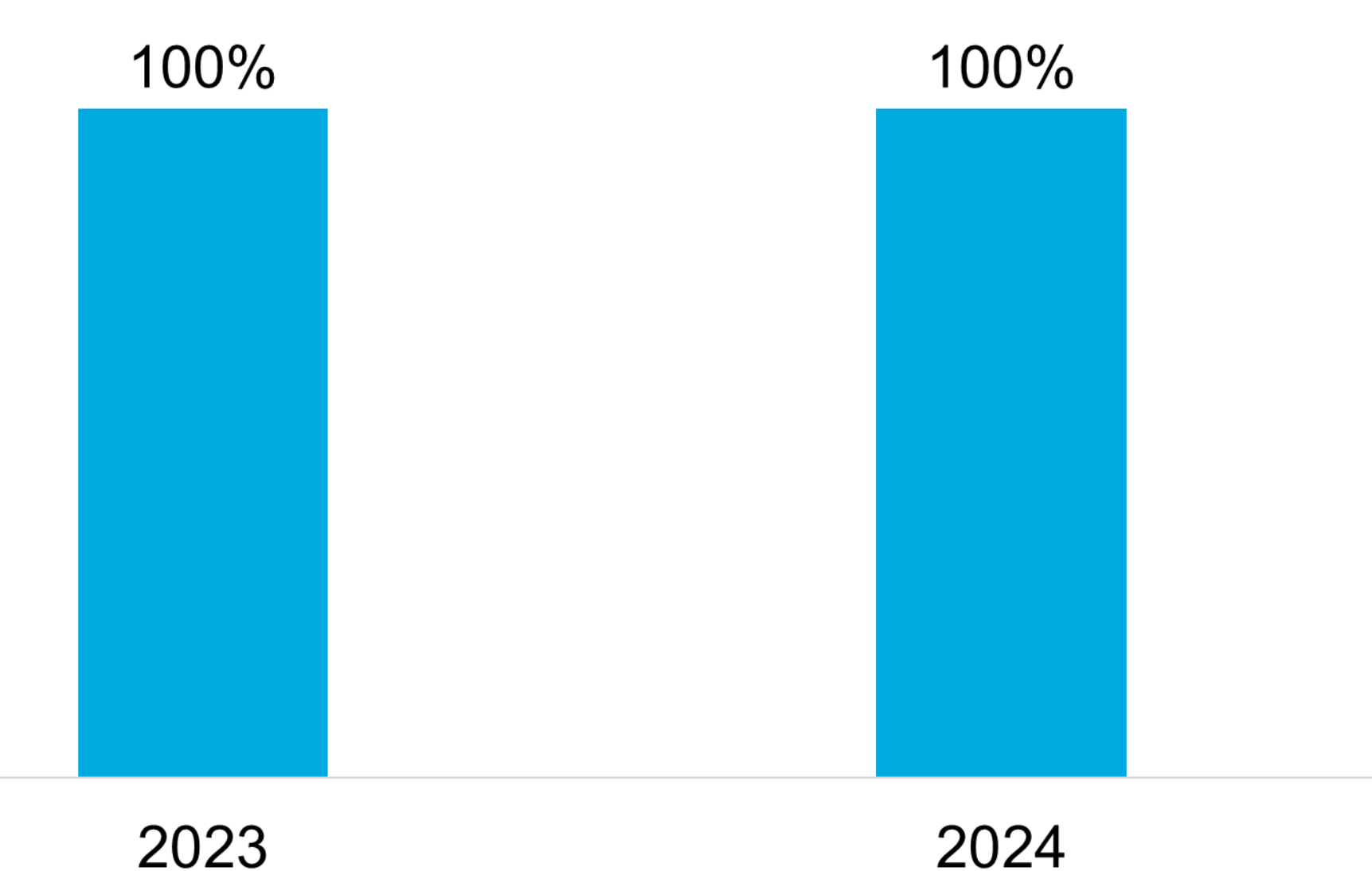
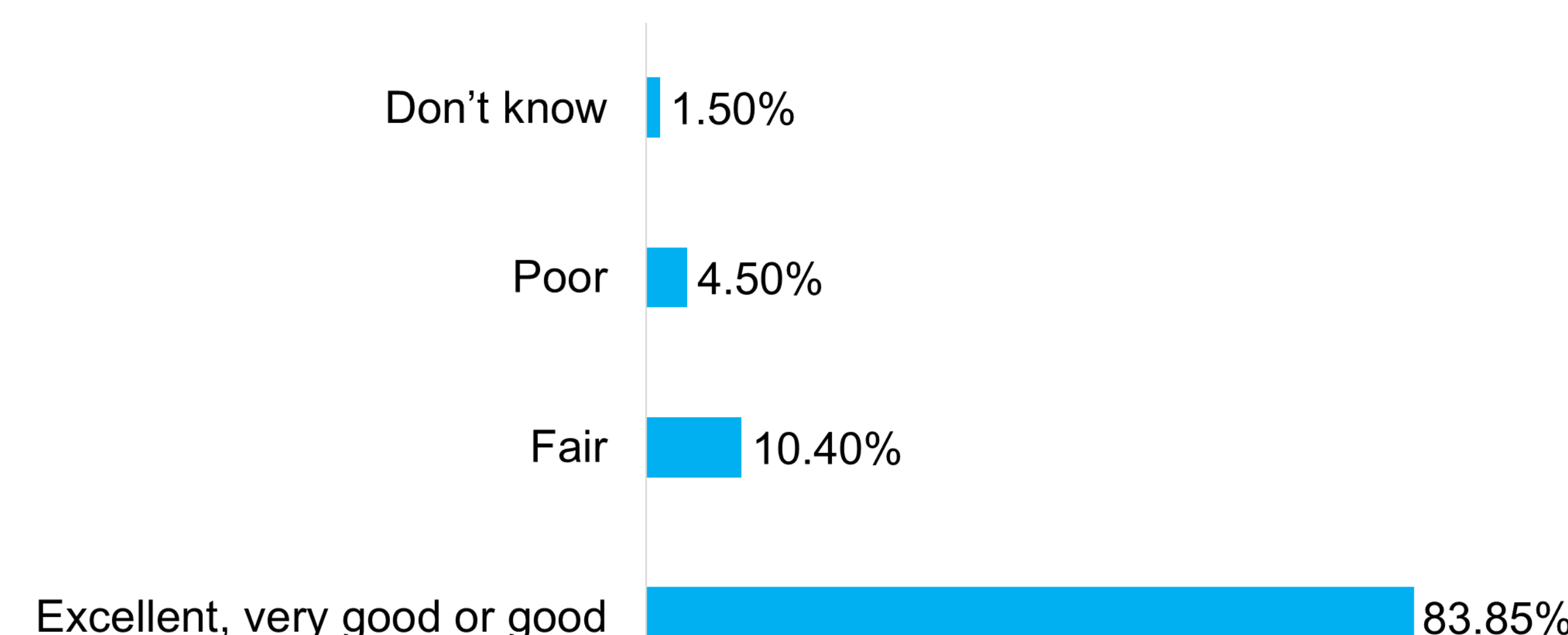


Figure 4: Patient reported Quality of Life (n=67)



Discussion

- Pharmacists exclusively handled all aspects of clinical management and delivery coordination for mavacamten.
- The integrated HSSP model facilitated clinical continuity and close collaboration between the in-clinic and specialty pharmacists ensuring compliance with the strict REMS requirements
- Continued utilization of the medication delivery communication channel ensured 100% compliance with the dispensing checklist.
- The high-touch integrated HSSP model promoted adherence, the average PDC and MPR was 0.96 exceeding the benchmark and stretch goal for both measures.
- The frequent monitoring and 100% compliance with the REMS requirements resulted in 83.85% of patients reporting an excellent, very good or good quality of life.

Limitations

- Our integrated dispensing and clinical documentation platform enables seamless tracking of medication delivery which may not be feasible at other health systems.

Future Directions

- Expand access of mavacamten to external patients being seen by providers outside of the HSSP.
- Create a specialty pharmacy center of excellence for hypertrophic obstructive cardiomyopathy by working in close collaboration with providers, payers, regulators, and patients.

Conclusion

An optimized clinical and dispensing workflow at an integrated HSSP can ensure compliance with strict REMS requirements with optimal patient outcomes.

References

Justin Bivona, Alec Fouche, Lauren Voide et al., Optimizing Mavacamten Clinical and Dispensing Practices at an Integrated Specialty Pharmacy. Presented at NASP Annual Meeting 2024, Nashville, TN.

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